Hospital Policy & Standard Operating Procedures Manual

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1. Purpose and Scope

This manual sets out the mandatory policies for inpatient care, discharge, bed management, insurance and claims, infection control, patient rights and grievance redressal at Sunrise Multispeciality Hospital (SMH). It applies to all 300 inpatient beds across nine clinical departments, all consultants, resident doctors, nursing staff, TPA/insurance desk, billing and front-office staff. Policies align with NABH 5th Edition standards.

2. Admission Policy

2.1 Emergency admissions

- Every patient arriving at Emergency must be triaged within **10 minutes** using the 5-level colour triage (Red, Orange, Yellow, Green, Blue).
- Target door-to-admission (ER wait) time: **under 30 minutes** for Red/Orange and under 60 minutes for all others. Wait above 90 minutes must be logged as an incident.
- No emergency patient may be refused or delayed for want of deposit. Stabilisation comes first; financial counselling follows.

2.2 Elective admissions

- Elective admissions are scheduled through the Admission Desk 24-72 hours in advance with a written cost estimate.
- For insured patients, pre-authorisation must be raised with the TPA/scheme **at least 48 hours before** an elective admission. Emergency pre-authorisation must be raised within **24 hours** of admission.
- Elective surgeries in Cardiology, Nephrology and Neurology are rescheduled when department occupancy is above 90% (see Section 4).

3. Discharge Policy (Discharge Before Noon)

- Discharge planning begins at admission; the expected date of discharge (EDD) must be recorded within 24 hours of admission.
- Target: **60% of planned discharges completed before 12:00 noon**. Discharges after 18:00 require Duty Medical Officer approval because late-evening discharges show higher readmission.
- Target time from discharge order to patient exit: **3 hours** for cash patients and **4 hours** for insured patients.
- Every discharge summary must contain diagnosis, procedures, medicines with dose and duration, danger signs, diet, follow-up date and a 24x7 helpline number (0422-400-7000), in English and Tamil.

3.1 Medication Reconciliation

A clinical pharmacist must complete medication reconciliation for every patient discharged on five or more medicines, and for all Cardiology, Nephrology, Pulmonology and Neurology patients. The pharmacist explains each medicine to the patient or caregiver using the teach-back method.

4. Readmission Prevention Programme (Care Transitions)

- A **readmission** is any unplanned admission to SMH within 30 days of a previous discharge. Planned readmissions (chemotherapy cycles, staged surgery) are excluded.
- At discharge, every patient receives a readmission risk score from the hospital's predictive model: **Low (<10%)**, **Medium (10-25%)**, **High (>25%)**.
- **All** patients must receive a nurse follow-up call **within 48 hours** of discharge. The call checks symptoms, medicine availability and adherence, wound status and confirms the follow-up appointment.
- High-risk patients additionally receive: a 7-day follow-up OPD visit (free consultation), a day-14 phone call, and tele-consultation access for 30 days.
- Patients living more than 50 km away are offered tele-follow-up in place of the physical 7-day visit.
- Every readmission within 7 days is reviewed by the department head within 72 hours (root-cause review), and summarised at the monthly Quality Committee.

5. Bed Management and Capacity Escalation

Trigger	Level	Action
Hospital occupancy > 85%	Amber	Bed manager huddle at 08:30 and 14:00; expedite discharges; open transit lounge
Department occupancy > 90% for 3 consecutive days	Red	Defer elective admissions in that department; borrow beds from low-occupancy wards (OBG, General Medicine) with nursing support
ICU occupancy > 90%	Red	Intensivist reviews step-down candidates every 6 hours; Medical Superintendent informed; no elective cases needing post-op ICU
ICU occupancy 100%	Black	Activate referral network; inform patients' families; CEO notified

Bed turnaround (cleaning and readiness after discharge) must not exceed **60 minutes**. The housekeeping supervisor is notified automatically from the HIS when a discharge is completed.

6. Insurance, TPA and Claims Policy

- Accepted payers: Private health insurance via TPA, Corporate tie-ups, ESI, Ayushman Bharat PM-JAY and Tamil Nadu Chief Minister's Comprehensive Health Insurance Scheme (CMCHIS).
- Final claim documents must be submitted **within 3 working days** of discharge. Any claim submitted later than 7 days requires a written explanation from the TPA desk lead.
- Mandatory claim checklist: pre-auth approval, ID and scheme card, discharge summary, itemised bill, investigation reports, implant stickers and invoices (if any), and signed consent forms.
- Room-rent eligibility must be explained to every insured patient at admission to avoid proportionate deductions.
- For PM-JAY and CMCHIS, the package code booked must match the procedure performed; any change requires an enhancement request before discharge.
- Rejected or partially approved claims are re-appealed within **15 days**. Claims rejected for 'incomplete documents' are reviewed weekly by the Revenue Cycle Manager.

7. Billing and Financial Counselling

- Cost estimates are given in writing; final bills more than 15% above the estimate need the Billing Manager's approval and an explanation to the patient.
- Cash patients may request an interim bill at any time. Discounts above 10% need Medical Superintendent approval.
- Refunds for excess deposit are processed within 7 working days.

8. Infection Prevention and Control

- Hand hygiene compliance target: 90% (audited monthly by the infection control nurse).

- Surgical site infection (SSI) target below 1.5%; catheter-associated UTI below 2 per 1,000 catheter days.
- Antibiotic use follows the hospital antibiogram; restricted antibiotics need Infectious Disease consultant approval.

9. Patient Rights, Feedback and Grievance Redressal

- Patients have the right to information about diagnosis, treatment, cost, and to a second opinion and copies of medical records (within 72 hours of request).
- Every discharged patient is invited to a feedback survey by SMS. Complaints are acknowledged within 24 hours and closed within 7 days.
- Leaving Against Medical Advice (LAMA): the doctor must counsel the patient, document risks, and obtain a signed LAMA form. LAMA patients still receive the 48-hour follow-up call.
- Grievance officer: Patient Relations Manager, Ground Floor, Block A; email grievance@sunrisehospital.example.

10. Data Privacy and AI Use

- Patient data is handled in line with the Digital Personal Data Protection Act, 2023. Only authorised staff may access the HIS.
- AI tools (readmission model, staff chatbot) support but never replace clinical judgement. The staff chatbot answers only from approved hospital documents and must show its source.
- Model performance is reviewed quarterly by the Quality and IT committees.

11. Key Quality Indicators and Targets

Indicator	Target
30-day readmission rate	< 10%
Average length of stay	< 5.8 days
Bed occupancy	75-80%
ER triage time	< 10 min
Claim rejection rate	< 5%
48-hr follow-up call coverage	> 90%
LAMA rate	< 2%
Net Promoter Score	> 40